

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Mebendazole 100mg chewable tablets/oral suspension for the treatment of Threadworm Infection (*Enterobius vermicularis*)

Summary of NICE / NICE CKS Guidelines for Threadworm Infection (*Enterobius vermicularis*)

Threadworm (*Enterobius vermicularis*) - Overview

Threadworms (pinworms) are the most common parasitic infection in children in developed countries. Infection is highly contagious and commonly affects multiple family members.

Clinical Presentation

- Perianal itching (especially at night) is the cardinal symptom; may also occur in females in vaginal area.
- Itching may disturb sleep, causing restlessness or irritability.
- Visible worms in stool or perianal region (appear as small white threads).
- Often diagnosed incidentally or via parental observation.

Transmission and Risk

- Transmission occurs via fecal-oral route; eggs ingested and hatch in small intestine.
- Eggs become infectious within 4-6 hours of passage.
- Highly contagious in household settings; all household members should be treated simultaneously.

Treatment Strategy

- Single dose mebendazole 100mg, repeat after 2 weeks to cover reinfection from newly hatched eggs.
- Combined pharmacological and hygiene measures are essential for effective eradication.
- Over-the-counter treatments available; PGD supply provides clinical access and guidance.

Patient Group Direction

for the supply/administration of

Mebendazole 100mg chewable tablets/oral suspension

for the treatment of

Threadworm Infection (*Enterobius vermicularis*)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC

	Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. - All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines - All users must previously have used PGDs to supply medication - Must work in compliance with the SOPs of their own employer - All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	- All users must be up to date with CPD requirements and appraisal - All users must be up to date with MHRA and safety alerts with regards to medical products - All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of threadworm (<i>Enterobius vermicularis</i>) infection in children and adults
Inclusion criteria	Children aged 2 years and above and adults Confirmed or clinically suspected threadworm infection Parental/guardian consent (for children <16 years) Able to take oral medication
Exclusion criteria	Age less than 2 years (hygiene measures only; refer to GP) Known hypersensitivity to mebendazole or excipients Pregnancy (mebendazole is teratogenic in animal studies; avoid during first trimester; use only if essential in 2nd/3rd trimester) Severe hepatic or renal impairment (use with caution)
Cautions	Breastfeeding: mebendazole has minimal systemic absorption; generally considered safe, but use with caution and only if benefit outweighs risk. In pregnant women (especially first trimester), use hygiene measures alone; seek medical advice. In children with suspected malabsorption or intestinal disease, consider referral to GP. Concurrent use of other medications: mebendazole may interact with cimetidine; assess clinically.
Actions if patient is excluded or	Advise on alternative treatment options and how these can be

declines treatment	accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.
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Details of the medicine

Name, form and strength of medicine	Mebendazole 100mg chewable tablets or oral suspension (5mg/mL)
Legal category	POM / P / GSL — mebendazole 100 mg chewable tablets and oral suspension (5 mg/mL) are available as P/GSL (OTC) packs and as POM packs of the same strength and form. Supply under this PGD may be made from either legal category.
Storage	Store at room temperature, protected from light and moisture. Keep in original container.
Route/method of administration	Oral (chewed or swallowed whole with food)
Dose and frequency	Single dose: 100mg (one tablet or 20mL suspension) Repeat dose: 100mg after 2 weeks Dose is the same for all ages ≥ 2 years (not weight-dependent)
Quantity to be administered and/or supplied	2 tablets or 40mL suspension (one dose + repeat dose pack)
Maximum or minimum treatment period	Single initial dose, with repeat dose at 2 weeks
Adverse effects	Abdominal pain and gastrointestinal discomfort (occasionally) Diarrhoea (uncommon) Nausea (rare) Rash and pruritus (rare, may relate to dying parasites) Severe hypersensitivity reactions (very rare)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken

	supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Provide the patient information leaflet (PIL) supplied with the medication. Explain that threadworm infection is common and treatable, and that strict hygiene measures are essential to prevent reinfection.
Follow-up advice to be given to patient or carer	Treat all household members at the same time with a single dose, even if asymptomatic. Strict hygiene measures must be followed during treatment and for 2-3 weeks after the initial dose: handwashing after toilet use, before meals, and on waking; keep fingernails short to reduce egg transmission; shower or bath each morning to remove eggs from the perianal area. Change and wash bedding, nightclothes, and underclothes daily in hot water. Vacuum carpets and clean household surfaces, especially in bedrooms and bathrooms. Avoid scratching affected areas to prevent ingestion of eggs and reinfection. Consider a second dose at 2 weeks to eliminate any parasites that hatch from eggs laid after the first dose. If symptoms persist beyond 2 weeks after the second dose, contact the GP for further advice. Maintain general hygiene measures (handwashing, food hygiene) to prevent reinfection in future.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Management of Threadworm Infection
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and

administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date

Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
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Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025
002	Legal category updated to reflect POM availability of mebendazole (same strength and form as P/GSL packs)	13th July 2026